

Learner handout – Feminizing hormones

Medication	Formulations	Principal effects	<u>Contraindications,</u> side effects	Lab monitoring

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Estradiol <i>*No evidence for progesterone therapy</i>	Oral, transdermal patch, IM/SQ injection	Breast development, fat redistribution, reduction body hair, arrest scalp hair loss	<u>Hemodynamically significant PE/DVT*</u> , <u>hormone receptor positive malignancy</u> Erectile dysfunction, hyperprolactinemia	Total estradiol at 3 months, 6 months, and then yearly Total testosterone at 3 months , 6 months, then yearly Goal= ♀ physiologic range
Spironolactone	Oral	Reduction body hair, secondary male sex characteristics	<u>Hyperkalemia</u> Orthostasis, polyuria, AKI Erectile dysfunction	BUN/Cr/K 3 months, 6 months, then yearly
5 alpha reductase inhibitors (finasteride, dutasteride)	Oral	Reduction male pattern baldness	Orthostasis Low libido Erectile dysfunction	None <i>May make T higher</i>
GnRH agonists (leuprolide, etc.)	IM, surgical implementation	Repress secondary sex characteristics/ puberty delay	Osteoporosis, increased cardiovascular/CVA events	Estradiol/testosterone levels per above FSH/LH in select circumstances

Learner handout - Masculinizing hormones

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Testosterone (bioidentical)	Gel, IM/SubQ, Patch	Facial hair, virilizing vocal changes, increased muscle mass, increased body hair, clitoral growth, cessation menses	<u>Active sex-hormone receptive cancer,</u> <u>pregnancy</u> Polycythemia, acne, frontal/temporal hairline recession, +/- hyperlipidemia, CV risk <i>*Note, testosterone NOT associated with worsened mood disorders when treating gender dysphoria</i>	Total testosterone and CBC at 3 months, 6 months, yearly Consider estradiol measurement Lipids, A1C based on USPSTF guidelines for males Goal testosterone level between dose = (400-700 ng/dL)